



DOAC Guideline in AF & VTE



Prepared by PRIC HSgB.

For internal circulation only. To consult physician if needed.

CHECKLIST

DABIGATRAN

For SPAF: Usual dose^[1,4]

150mg BD

Dosing Adjustment^[1,4]

- 110mg BD if fulfill **ANY** of the following criteria:

- i) Patients aged ≥80 years
- ii) Concomitant use of Verapamil / Amiodarone

**Please refer to bottom of the table for further dosing consideration*

For VTE: Treatment / Prevention^[1,4]

PARENTERAL ANTICOAGULANT for at least 5 days

THEN

150mg BD

OR

110mg BD if fulfill any of the following risk factors

- i) Patients aged ≥80 years
- ii) Concomitant use of Verapamil / Amiodarone
- iii) High bleeding risk **AND** CrCl: 30-50 ml/min

**Please refer to bottom of the table for further dosing consideration*

RENAL FUNCTION

<30 ml/min: Contraindicated^[1,4]

APIXABAN

For SPAF: Usual dose^[2]

5mg BD

Dosing Adjustment^[2]

- 2.5mg BD if fulfill **TWO** of the following criteria:

- i) Patients aged ≥80 years
- ii) Body weight ≤60kg
- iii) Serum Creatinine ≥133 umol/L

- 2.5mg BD if CrCl 15-29ml/min **OR** on regular dialysis^[16].

For VTE: Treatment^[2]

10mg BD for 7 days

THEN

5mg BD

Prevention of Recurrent VTE^[2]

2.5mg BD (after at least 6 months of therapeutic dose of anticoagulant)

15 – 29 ml/min : Use with caution^[2]

<15 ml/min : Contraindicated ϕ

HD/CAPD : 2.5mg BD for SPAF only ^[16]

RIVAROXABAN

For SPAF: Usual Dose^[3]

20mg OD

Dosing Adjustment^[3]

- 15 mg OD if CrCl 15-49 ml/min

For VTE: Treatment^[3]

15mg BD for 21 days

THEN

20mg once daily

Prevention of Recurrent VTE^[3]

10 mg OD (after at least 6 months of therapeutic dose of anticoagulant)

15 – 29 ml/min : Use with caution^[3]

<15 ml/min : Contraindicated

EDOXABAN

For SPAF: Usual Dose^[14]

60mg OD

Dosing Adjustment^[14]

- 30mg OD if fulfill **ANY** of the following criteria:

- i) CrCl 15 – 50ml/min
- ii) Body weight ≤60kg
- iii) Concomitant use of cyclosporin, dronedarone, erythromycin, ketoconazole

For VTE: Treatment / Prevention^[14]

PARENTERAL ANTICOAGULANT for at least 5 days

THEN

60mg OD

OR

30mg OD if fulfill any of the following risk factors

- i) CrCl 15 – 50ml/min
- ii) Body weight ≤60kg
- iii) Concomitant use of cyclosporin, dronedarone, erythromycin, ketoconazole

<15 ml/min: Contraindicated^[14]

>95 ml/min: Not recommended for SPAF due to increased risk of ischemic stroke^[15]

* **FOR DABIGATRAN:** May consider adjustment to 110mg BD based on individual thromboembolic and bleeding risk assessment for the following groups:

i) Age: 75-80 years, ii) Moderate renal impairment (30 – 50ml/min), iii) Hx of gastritis, esophagitis or gastroesophageal reflux, iv) Patients with increased risk of bleeding

ϕ **FOR APIXABAN:** US-FDA approved the use of Apixaban in ESRF on regular dialysis based on PK studies for the indication of SPAF only. ^[7]

ϕ For AF patient with ESRF on regular HD/CAPD: T. Apixaban 2.5mg BD ^[16]

CHECKLIST	DABIGATRAN	APIXABAN	RIVAROXABAN	EDOXYABAN
HEPATIC FUNCTION	Child Pugh A : No dosage adjustment. Child Pugh B : Use with caution. Child Pugh C : Contraindicated. ^[4-5] <i>Not recommended when elevated liver enzymes >2 ULN^[1,4]</i>	Child Pugh A : No dosage adjustment. Child Pugh B : Use with caution. Child Pugh C : Contraindicated <i>Use with caution when elevated liver enzymes >2 ULN^[2,4,5]</i>	Child Pugh A : No dosage adjustment. Child Pugh B : Contraindicated Child Pugh C : Contraindicated ^[3-5] <i>No data for elevated liver enzymes >3 ULN^[6]</i>	Child Pugh A : No dosage adjustment. Child Pugh B : Contraindicated. Child Pugh C : Contraindicated. <i>No data for elevated liver enzymes >2 ULN & T.Bilirubin ≥1.5 ULN^[14]</i>
PREGNANCY & LACTATION	Not recommended. ^[1-5]			
WEIGHT CONSIDERATION <i>* Low body weight: ↑bleeding risk</i> <i>* Obesity: ↓ drug exposure</i>	Wt <50kg: Use with caution.^[1] BMI >40kg/m² or Wt >120 kg: Avoid use. Suggest to use alternative.^[5]	Wt <50kg (Use with caution) For SPAF: Adjust dose if other criteria are met. For VTE : No dosage adjustment. ^[2] BMI >40kg/m² or Wt >120 kg: Use with caution. Consult physician.^[5]	Wt <50kg: Use with caution.^[3, 8, 9] BMI >40kg/m² or Wt >120 kg: No dosage adjustment. ^[3, 9]	Wt ≤60kg: 30mg OD ^[14] BMI 40-49kg/m²^[15]: For SPAF: No dosage adjustment. For VTE : Avoid use.
ADMINISTRATION	Swallow whole. Do NOT break, chew or open capsules. ^[1]	Swallow whole or crush the tablet and suspend in 60mL of water / D5. ^[2]	Swallow whole or crush the tablet and suspend in 50mL of water / D5. To be taken with food. ^[3]	Swallow whole or crush the tablet and suspend in small amount of water. Stable up to 4 hours. ^[14]
PAEDIATRIC <i>Off-Label Use</i> <i>Please refer to References 10 – 12 for dosing recommendations</i> <i>*Initiate with a parenteral anticoagulant for at least 5 days</i>	<u>For VTE:Treatment / Prevention</u> ^[10-11] Oral pellet : 3 months to <12 years old ^[10] <i>(Not available in Malaysia)</i> Capsule : 8 to <18 years old ^[11] <i>The dosage forms are NOT interchangeable on a mg-for-mg basis due to differences in bioavailability.</i>	No safety & efficacy studies.	<u>For VTE:Treatment / Prevention</u> ^[12] From birth to <18 years old: Oral suspension : ≥2.6 kg <i>(Not available in Malaysia)</i> Tablet : ≥30 kg	No safety & efficacy studies.
CANCER-ASSOCIATED VTE <i>Off-Label Use</i>	No safety & efficacy studies.	Recommended. ^[13] <i>LMWH more preferred for gastrointestinal/genitourinary related cancer. ^[13]</i>	Recommended. ^[13] <i>LMWH more preferred for gastrointestinal/genitourinary related cancer. ^[13]</i>	Recommended. ^[13] <i>LMWH more preferred for gastrointestinal/genitourinary related cancer. ^[13]</i>
REVERSAL AGENT ^[15]	IV Idarucizumab 5g STAT Administer as two separate 2.5g doses with NO >15 mins apart. Alternative if Idarucizumab is not available: IV Octaplex 50 units/kg STAT (Max: 4000 units)	IV Andexanet alfa (Not available in HSgB) Alternative: IV Octaplex 25-50 units/kg STAT OR IV Octaplex 2000 units STAT	IV Andexanet alfa (Not available in HSgB) Alternative: IV Octaplex 25-50 units/kg STAT OR IV Octaplex 2000 units STAT	IV Andexanet alfa (Not available in HSgB) Alternative: IV Octaplex 25-50 units/kg STAT OR IV Octaplex 2000 units STAT
PHYSICAL APPEREANCE	 Pradaxa hard cap 150mg Pradaxa hard cap 110mg	 Eliquis FC Tab 5mg Eliquis FC Tab 2.5mg	 Xarelto FC Tab 20mg Xarelto FC Tab 15mg	 Lixiana FC Tab 60mg Lixiana FC Tab 30mg

References:

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